NCCP 00568 · clinician draft

Pembrolizumab + pemetrexed + carboplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: carboplatin-containing cycles 1–4

Active medicines: pembrolizumab + pemetrexed + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before SACT on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V8 row Reference course only: reconcile with pemetrexed rash prophylaxis and immunotherapy context before a single prescription is issued.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS carboplatin row does not specify an end day or PRN qualifier. Prescriber/pharmacy must specify duration and suitability.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms IM	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.

Additional phase-specific support

- High emetic risk is driven by carboplatin AUC5. Dedicated NCIS carboplatin row has no routine olanzapine.
- Do not combine rash and antiemetic dexamethasone courses without reconciliation.
- Carry immunotherapy alert card; do not wait until next cycle for possible immune toxicity.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required